

LEGISLATURE OF THE STATE OF IDAHO
Sixty-eighth Legislature Second Regular Session - 2026

IN THE HOUSE OF REPRESENTATIVES

HOUSE BILL NO. 759

BY HEALTH AND WELFARE COMMITTEE

AN ACT

RELATING TO PUBLIC ASSISTANCE; PROVIDING LEGISLATIVE INTENT; AMENDING SECTION 56-265, IDAHO CODE, TO REVISE PROVISIONS REGARDING PROVIDER PAYMENT; PROVIDING THAT CERTAIN ADMINISTRATIVE RULES CONTAINED IN IDAPA 16.03.26 SHALL BE NULL, VOID, AND OF NO FORCE AND EFFECT; AND DECLARING AN EMERGENCY.

Be It Enacted by the Legislature of the State of Idaho:

SECTION 1. LEGISLATIVE FINDINGS AND INTENT. The Legislature seeks to increase transparency for taxpayers and to support appropriation reductions for Medicaid rates paid to residential habilitation providers. Medicaid pays more than \$176.5 million in general funds to help people with disabilities live independently with residential habilitation and other home and community-based services. The Governor's recommendation calls for the Legislature to pursue policy changes to support the Department of Health and Welfare in achieving budget reductions for Medicaid, including an option to reduce rates for residential habilitation. In 2022, the Legislature appropriated funds pursuant to Chapter 252, Laws of 2022, to increase payment rates for these services to implement a new service array and budget tool associated with the *K.W. v. Armstrong* lawsuit, but these funds are no longer appropriate since a court order halted implementation of the new services and budget tool. To support ongoing accountability for taxpayer dollars and ensure the Legislature has the needed information to effectuate its appropriation responsibilities, this act requires the department to report how those funds are used.

SECTION 2. That Section 56-265, Idaho Code, be, and the same is hereby amended to read as follows:

56-265. PROVIDER PAYMENT. (1) Where there is an equivalent, the payment to medicaid providers:

(a) May be up to but shall not exceed one hundred percent (100%) of the current medicare rate for primary care procedure codes as defined by the centers for medicare and medicaid services; and

(b) Shall be ninety percent (90%) of the current medicare rate for all other procedure codes.

~~(2) Where there is no medicare equivalent, the payment rate to medicaid providers shall be prescribed by rule. All home and community-based services without a medicare equivalent rate shall be cost-surveyed annually with fifteen percent (15%) or more of responses being audited. The department shall use information from the cost surveys and other sources to evaluate rate adequacy. Payment rates shall be developed to include allocations to direct care worker wages, employee-related expenses, program-related expenses, and general and administrative costs.~~

1 (a) On an annual basis, providers are required to expend at least the
 2 amount allocated to direct care worker wages and employee-related ex-
 3 penses to these categories.

4 (b) Failure of the provider to meet the requirement in paragraph (a) of
 5 this subsection may result in a department-approved corrective action
 6 plan, closure of intake, or termination of the provider agreement.

7 (c) The department shall summarize this audited cost survey work by
 8 provider type and service in a publicly available report no later than
 9 December 31 of each calendar year.

10 (3) Notwithstanding any other provision of this chapter, if the
 11 services are provided by a private, freestanding mental health hospital
 12 facility that is an institution for mental disease as defined in 42 U.S.C.
 13 1396d(i), the department shall reimburse for inpatient services at a rate
 14 not to exceed ninety-one percent (91%) of the current medicare rate within
 15 federally allowed reimbursement under the medicaid program. The reimburse-
 16 ment provided for in this subsection shall be effective until July 1, 2021.

17 (4) The department shall, through the annual budget process, include
 18 a line-item request for adjustments to provider rates. All changes to
 19 provider payment rates shall be subject to approval of the legislature by
 20 appropriation.

21 (5) Notwithstanding any other provision of this chapter, the depart-
 22 ment may enter into agreements with providers to pay for services based on
 23 their value in terms of measurable health care quality and positive impacts
 24 to participant health.

25 (a) Any such agreement shall be designed to be cost-neutral or cost-
 26 saving compared to other payment methodologies.

27 (b) The department is authorized to pursue waiver agreements with the
 28 federal government as needed to support value-based payment arrange-
 29 ments, up to and including fully capitated provider-based managed care.

30 (c) Beginning with the 2024 performance period and for all future per-
 31 formance periods thereafter, federally qualified health centers and
 32 any organization owned and controlled by a federally qualified health
 33 center shall be exempt from any financial risk in value-based payment
 34 agreements created pursuant to this section.

35 (6) Medicaid reimbursement for critical access, out-of-state, and
 36 state-owned hospitals shall be as follows:

37 (a) In-state, critical access hospitals as designated according to 42
 38 U.S.C. 1395i-4(c) (2) (B) shall be reimbursed at one hundred one percent
 39 (101%) of cost;

40 (b) Out-of-state hospitals shall be reimbursed at eighty-seven percent
 41 (87%) of cost;

42 (c) State-owned hospitals shall be reimbursed at one hundred percent
 43 (100%) of cost; and

44 (d) Out-of-state hospital institutions for mental disease as defined
 45 in 42 U.S.C. 1396d(i) shall be reimbursed at a per diem equivalent to
 46 ninety-five percent (95%) of cost.

47 (7) The department shall equitably reduce net reimbursements for all
 48 hospital services, including in-state institutions for mental disease but
 49 excluding all hospitals and institutions described in subsection (6) of
 50 this section, by amounts targeted to reduce general fund needs for hospital

1 payments by three million one hundred thousand dollars (\$3,100,000) in state
2 fiscal year 2020 and eight million seven hundred twenty thousand dollars
3 (\$8,720,000) in state fiscal year 2021.

4 (8) The department shall work with all Idaho hospitals, including in-
5 stitutions for mental disease as defined in 42 U.S.C. 1396d(i), to establish
6 value-based payment methods for inpatient and outpatient hospital services
7 to replace existing cost-based reimbursement methods for in-state hospi-
8 tals, other than those hospitals and institutions described in subsection
9 (6) of this section, effective July 1, 2021. Budgets for hospital payments
10 shall be subject to prospective legislative approval.

11 (9) The department shall work with Idaho hospitals to establish a
12 quality payment program for inpatient and outpatient adjustment payments
13 described in section 56-1406, Idaho Code. Inpatient and outpatient adjust-
14 ment payments shall be subject to increase or reduction based on hospital
15 service quality measures established by the department in consultation with
16 Idaho hospitals.

17 SECTION 3. The rules contained in IDAPA 16.03.26, Department of Health
18 and Welfare, relating to Medicaid Plan Benefits, Section 051.; and Section
19 052., shall be null, void, and of no force and effect on and after July 1,
20 2026.

21 SECTION 4. An emergency existing therefor, which emergency is hereby
22 declared to exist, this act shall be in full force and effect on and after its
23 passage and approval.